

THE NUTRITION CLUB CLINIC

IBS Guide

A calm, practical introduction to eating well with irritable bowel syndrome.

Triggers differ from person to person — personalised guidance matters.

What's inside

Irritable bowel syndrome is a disorder of gut–brain interaction: the bowel looks normal but functions and signals abnormally, producing pain, bloating and unpredictable bowel habits. It is real, common and manageable. This guide covers the subtypes, the low FODMAP approach done properly, and the non-food factors that matter just as much.

1. What IBS is — and what it isn't
2. Tracking your own pattern
3. First-line changes to try for four weeks
4. The low FODMAP approach, done properly
5. The gut–brain axis, sleep and daily rhythm

How to use this guide

Work through it in order. Get a diagnosis and rule out red flags first, try the general first-line changes in section 3 for four weeks, and only move to the structured low FODMAP process in section 4 if you still need to — ideally with a dietitian.

1. What IBS is — and what it isn't

In IBS the nerves of the gut are hypersensitive and gut motility is dysregulated, so normal digestion is felt as pain, urgency or bloating. It does not damage the bowel, does not cause cancer, and does not shorten life — but it can significantly affect quality of life.

The subtypes

Subtype	Pattern	Typical priority
IBS-D	Diarrhoea predominant	Slowing transit, soluble fibre, identifying trigger foods
IBS-C	Constipation predominant	Fluid, soluble fibre, kiwifruit, magnesium, movement
IBS-M	Mixed — alternating	Regular meal timing and stress management first
IBS-U	Unclassified	Symptom-led approach

Red flags — see a doctor, these are not IBS

- Blood in the stool, or black tarry stools.
- Unintentional weight loss; fever; anaemia.
- Symptoms starting for the first time after age 50.
- Night-time symptoms that wake you from sleep.
- A family history of bowel cancer, coeliac disease or inflammatory bowel disease.
- Persistent vomiting, or a mass felt in the abdomen.

Get coeliac disease excluded

It shares many symptoms with IBS and the blood test only works while you are still eating gluten. Do not cut gluten out before being tested.

2. Tracking your own pattern

IBS triggers are individual. Two weeks of honest tracking is worth more than any generic list of ‘bad foods’.

Keep a simple log

Record	Detail
Time and food	What you ate and drank, roughly how much, and how it was cooked
Symptoms	Pain, bloating, wind, urgency — rated 0–10, with the time they started
Stool	Bristol stool type 1–7 and frequency
Context	Sleep, stress level, menstrual cycle day, exercise, medicines

Look for patterns that repeat at least three times before blaming a food. Symptoms often lag hours behind the meal, and stress or a poor night's sleep can be the real driver on a bad day.

Beware over-restriction

The biggest risk in IBS is ending up on a tiny list of ‘safe’ foods. That damages nutrition, the gut microbiome and your relationship with eating — and often makes symptoms worse over time. The goal is the widest diet you can tolerate, not the narrowest.

3. First-line changes to try for four weeks

These general measures resolve or substantially improve symptoms for many people, without any complicated elimination.

- 1 **Regular meals.** Three meals at roughly the same times daily; don't skip and then overeat.
- 2 **Eat slowly and chew well.** Rushed eating and talking while eating swallows air and worsens bloating.
- 3 **Moderate portion sizes.** Large meals stretch the gut and trigger pain in a hypersensitive bowel.
- 4 **Cut back caffeine** to fewer than three drinks a day — it stimulates the colon, especially in IBS-D.
- 5 **Reduce alcohol and fizzy drinks.** Both are common aggravators.
- 6 **Limit fatty and fried food** — fat increases gut contractions after meals.
- 7 **Limit spicy food** if chilli reliably triggers you; capsaicin activates pain receptors in the gut.
- 8 **Drink 8–10 cups of fluid daily**, mostly water.
- 9 **Reduce sorbitol, mannitol and xylitol** — sugar-free gum, mints and diet products are frequent hidden triggers.
- 10 **Adjust fibre to your subtype.** Soluble fibre (oats, psyllium, chia, peeled fruit) helps both constipation and diarrhoea; excess insoluble bran often makes IBS worse.

Two things with decent evidence

Peppermint oil capsules (enteric-coated) can ease abdominal pain and spasm. **Soluble fibre supplements** such as psyllium help both IBS-C and IBS-D — start with a small dose and build up slowly with plenty of water. Check with your pharmacist first.

4. The low FODMAP approach, done properly

FODMAPs are short-chain carbohydrates that are poorly absorbed, ferment in the colon and draw in water — producing gas, bloating and altered bowel habit in sensitive guts. Around 70% of people with IBS improve on a low FODMAP diet, but it must be done in three phases. It is **not** a permanent diet.

FODMAP group	Common sources	Lower-FODMAP swaps
Fructans	Wheat, rye, onion, garlic, leek	Sourdough spelt, oats, rice, garlic-infused oil, green tops of spring onion
GOS	Chickpeas, lentils, kidney beans, soy	Small tinned and rinsed portions, firm tofu, peanuts
Lactose	Milk, soft cheese, yoghurt, ice cream	Lactose-free milk, hard cheese, lactose-free yoghurt
Excess fructose	Honey, mango, apple, high-fructose syrup	Berries, citrus, kiwi, banana (firm), maple syrup
Polyols	Stone fruit, mushrooms, cauliflower, sorbitol/xylitol	Grapes, melon, carrot, courgette, sugar-sweetened mints

The three phases

- 1 Restriction — 2 to 6 weeks.** Reduce all high FODMAP foods. If there's no improvement by six weeks, stop: FODMAPs are not your issue.
- 2 Reintroduction — 6 to 10 weeks.** Test one FODMAP group at a time over three days, in increasing amounts, with a settled gap between challenges. This identifies your specific triggers and your personal threshold.
- 3 Personalisation — ongoing.** Rebuild the broadest possible diet, avoiding only what you genuinely react to, in the amounts that matter.

Please do this with a dietitian if you can

Phase 1 is nutritionally restrictive and hard to shop for; phase 2 is where the real value lies and where most self-directed attempts stall, leaving people stuck on a needlessly limited diet for years.

5. The gut–brain axis, sleep and daily rhythm

The gut has its own nervous system in constant two-way conversation with the brain. This is why stress produces genuine physical gut symptoms, and why gut symptoms produce anxiety — a loop worth interrupting from both ends.

What actually helps

- **Gut-directed hypnotherapy** — surprisingly strong evidence, comparable to dietary change for many.
- **Cognitive behavioural therapy** for IBS, including app-based programmes.
- **Diaphragmatic breathing** — five minutes before meals shifts you into 'rest and digest' mode.
- **Regular moderate exercise** — walking, yoga and swimming improve symptoms and transit.
- **Consistent sleep** — poor sleep measurably increases next-day gut sensitivity.
- **Toilet routine** — respond to the urge, don't strain, and use a footstool to raise your knees.

Probiotics

Results are mixed and strain-specific. If you want to try one, use a single product for at least four weeks and judge honestly; if there's no benefit, stop rather than adding another.

Flare-up plan

- 1 Return to simple, plain, well-tolerated meals for a few days — don't panic-restrict everything.
- 2 Keep fluids up; warm drinks and a heat pack on the abdomen help cramping.
- 3 Gentle walking rather than intense exercise.
- 4 Prioritise sleep and lower your load for a few days where you can.
- 5 Look back at your log for what preceded it — often it's stress or sleep, not a food.
- 6 If a flare lasts more than two weeks or changes in character, contact your doctor.

Important note

This guide is general nutrition education only. It is not medical advice, a diagnosis, or a treatment plan, and it cannot account for your personal history, laboratory results, allergies or medicines. Always speak to your doctor, dietitian or pharmacist before making significant changes to your diet, supplements or activity — especially if you are pregnant or breastfeeding, have kidney, liver or heart disease, an eating disorder, or take prescription medication. If you experience severe or sudden symptoms, seek urgent medical care. IBS is a diagnosis of exclusion — conditions such as coeliac disease, inflammatory bowel disease and bowel cancer must be ruled out by a doctor before assuming symptoms are IBS.

Questions or want a personalised plan?

This guide covers general principles. A one-to-one consultation with The Nutrition Club Clinic lets us tailor portions, meal timing, food preferences, budget and any medications or conditions to you.

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